

# Inclusion and exclusion criteria

Eligibility framework for the Medicare review in progress

Adopted scope: 16 Sep 2026 | Proposed operational screening: 21 Sep 2026

## ADOPTED SCOPE

Cisgender women and men; retain eligible Medicare beneficiaries younger than 65.  
Original Medicare, Medicare Advantage and Part D; extract eligible findings separately.  
Coverage, payment values, patient costs and access to clinically indicated care.

## INCLUDE IF ELIGIBLE

Proposed empirical screening criteria

### A Patient evidence

Original research with a relevant sex comparison or a clinical-need/access question within an eligible sex-linked indication or population. Population eligibility and Medicare results must be established.

### B Service valuation evidence

Original research on Medicare services/codes with a defined sex-related valuation question. Assess clinical and resource comparability. Code-only results require no individual patient-identity assessment.

An opposite-sex comparator is not required; assess clinical need independently of coverage.

## EXCLUDE FROM PRIMARY

Adopted population / indication exclusions

Transgender-focused patient studies or gender-affirming-care indications.

Combined mixed-population estimates with no separable eligible cisgender results.

Retain the citation and exclusion reason.  
Assess separable results independently.

## AWAITING INFORMATION

Retain as unresolved; no automatic exclusion

Male/female sex recorded, identity unreported: assess full text and clarify where feasible. Do not infer cisgender identity.

Unclear eligibility or insufficient reporting: retain pending further information.

Unretrieved reports are logged separately.

## RETAIN SEPARATELY AS CONTEXT

Policy, formulary and descriptive code sources: assess indication, authority and dates; no patient-identity test. Empirical code studies use the service-valuation route above.

Clinical reviews/efficacy, physician-gender research and other-payer evidence.  
General intervention/health IT studies without the required equity or need question.  
No relevant Medicare empirical finding or review outcome: outside primary eligibility.

## CLASSIFY CARE AND THE SUBJECT OF EACH COMPARISON

Shared systems | reproductive anatomy | reproductive physiology | mixed/site-dependent  
Patient sex, physician gender and service anatomy remain distinct analytical variables.

Retain null and opposing findings. Language or full-text access alone is not an exclusion.

No selection counts or completed decisions are shown. Final eligibility assessment is pending.

Sources: REVIEW\_SCOPE.md and draft protocol sections 2, 3 and 6. Figure prepared 21 Sep 2026.